TABLE 1: Patient-Related Criteria

Criteria	Options	Impact on Surgery
Age	Child / Adult / Elderly	Child = No mesh. Elderly = Open under local
Sex	Male / Female	Female = Laparoscopic better
BMI	Normal / Obese	Obese = Laparoscopic better
Anesthesia Fitness	Fit for General / Not fit	Not fit = Only Open under Local
Activity Level	Heavy worker / Athlete / Sedentary	Heavy worker = Laparoscopic for fast recovery
Previous Lower Abdominal Surgery	Yes / No	Yes = Open is safer

TABLE 2: Hernia-Related Criteria

Criteria	Options	Impact on Surgery

Criteria	Options	Impact on Surgery
Side	Unilateral / Bilateral	Bilateral = Laparoscopic
Type	Primary / Recurrent	Recurrent after Open = Laparoscopic. Recurrent after Lap = Open
Size	Small <1.5cm / Medium / Large >3cm	Large / Scrotal = Open
Status	Reducible / Incarcerated / Strangulated	Strangulated = Emergency Open, No Laparoscopic
Type Anatomy	Indirect / Direct / Femoral	Femoral = Laparoscopic

TABLE 3: Surgical Method Choice

Surgical Method	When to Choose It
Open Mesh – Lichtenstein – Gold Standard	First time, Unilateral, Large hernia, Emergency, Can't have General anesthesia, Low cost needed
Laparoscopic – TEP / TAPP	Bilateral, Recurrent after open, Female, Obese, Young active patient

Open Non-Mesh – Shouldice / Herniotomy	Children, Infected / Strangulated bowel field where mesh is dangerous, Patient refuses mesh